

RETINAL EDGE TRIAGE

Explainable AI-Assisted Diabetic Retinopathy Screening for Rural Healthcare
Official Clinical Triage Report | Generated: 02-Sep-2026 15:01 UTC

Patient ID: RET-2026-987069	Full Name: parvathi	Age / Gender: 46 / Female
Screening ID: SCR-2026-000017	Location: PHC Rampur, Rural District	Diabetes Type / Duration: Type 1 (5 years)

1. Retinal Image Acquisition & Quality Gate

Eye Evaluated: Right Eye	Image Source: Fundus Camera	Quality Score: 64.2%	Quality Status: BORDERLINE
Focus / Blur: 16.7%	Illumination: 49.6%	Vessel Visibility: 76.1%	Field of View: 98.0%

2. AI Screening Assessment (APTOS 2019 & IDRI_D Models)

DR Severity Grade:	Grade 0 — No DR	Referable DR:	NO (Routine Follow-up)
AI Confidence:	97.0%	DME Risk Status:	Possible DME detected (Multiple hard exudates identified within parafoveal region, indicating potential macular edema risk.)
Detected Lesions (IDRI _D):	Microaneurysms: 25 Hemorrhages: 9 Hard Exudates: 16 Soft Exudates: 3	Model Version:	Demo DR Classifier (APTOS 2019 Arch) v1.0

3. Human-in-the-Loop Ophthalmologist Adjudication

Doctor Decision:	ACCEPTED	Reviewing Doctor:	Dr. Ophthalmologist
Final Grade Assigned:	Grade 0	Override Justification:	None
Clinical Notes:	Clinical findings are consistent with non-proliferative diabetic retinopathy.	HCW Instructions:	None

4. Risk Triage & Clinical Referral Plan

Risk Category:	LOW RISK	Referral Priority:	ROUTINE
Referral ID:	REF-2026-00009	Recommended Facility:	District Eye Hospital, Bareilly
Triage Reason:	Low Risk: No referable diabetic retinopathy. Routine annual screening advised.	Follow-up Schedule:	12 Months

STATUTORY CLINICAL NOTICE: This report is generated by an artificial intelligence screening triage prototype (Retinal Edge Triage). AI predictions DO NOT constitute a definitive medical diagnosis. Final clinical interpretation and management must always be performed by a qualified ophthalmologist. Compliant with rural screening clinical governance principles.